THE HAND-OUT SHELF · LICENSED FOR MORE THAN ONE PERSON

The Organisation Pack

TEAM LICENCE · £95

Seventeen printable tools and all twenty-three Pond Guides, in one file, with a licence to print them for every person you support. Cards to hand over instead of explaining. Sheets a person fills in about themselves and gives to you. Pages for getting through the meeting, for the day to day, and for the weeks after an assessment.

Everything here was written for the person it is about, not about them. Hand it over as it is.

LICENSED TO

_____

The licence

> Plain English, because a licence you cannot understand is not one you can keep. The short version: print as much as you like for the people you support; don't sell it, don't post the file publicly.

WHO THIS COVERS

One team of up to ten members of staff, at one organisation.

For example a classroom team, a small clinic, a ward, a department's ND network. If you are not sure which tier you are, the honest guess is fine — nobody is checking badges — and you can move up later by paying the difference.

WHAT YOU MAY DO

- Print any page, as many times as you need, for as long as you like. The licence does not expire.
- Hand printed copies to the people you support, their families and carers, and anyone else who needs one. There is no limit on how many people receive a sheet.
- Use the pages in sessions, lessons, inductions, assessments, care plans and training within the licensed team, service or organisation.
- Keep the file on the licensed staff's own devices and shared drives.
- Stamp or write your organisation's name on printed copies. Please leave the Frog Logic mark where it is.

WHAT YOU MAY NOT DO

- Sell the pack, any page of it, or anything made from it.
- Put the file, or any page of it, on a public website or in a public post.
- Pass the file to another organisation. They can buy their own; every sheet is also sold singly for personal use at froglogic.co.uk.
- Present the pages as your own work or remove the attribution.

WHAT YOU GET FOR THE NEXT TWELVE MONTHS

When a new sheet joins the pack, or one of these is improved, an updated file is sent to the email address that bought it. After twelve months the file you have stays yours forever; it just stops updating.

IF SOMETHING HERE IS WRONG FOR YOUR SETTING

Write to hello@froglogic.co.uk. Wording that does not fit a particular service gets changed, not defended.

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THE POND GUIDES

The Pond Guides — complete set 34

Page numbers count this cover and licence. Every tool is A4, designed to print in black and white on an office printer, and most are one or two sides. The Pond Guides are two pages each; print a single guide by its page range rather than the whole set.

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Access Needs Cards

HAND ONE OVER INSTEAD OF EXPLAINING

For appointments, new workplaces, shops, or any moment where saying it out loud is the hardest part. Print, cut, keep a few in your wallet.

I communicate better in writing

Please message or email me rather than calling. I'll reply properly.

FROG LOGIC

I need a moment to process

I'm not ignoring you — give me a few seconds and I'll answer.

FROG LOGIC

Please don't touch me without asking

Not personal. I'll tell you if it's okay.

FROG LOGIC

I may not make eye contact

I'm still listening. It's easier for me this way.

FROG LOGIC

Please give instructions in writing

I lose things said out loud, and I'd rather get it right.

FROG LOGIC

I may need to step outside

If I leave suddenly, I'm regulating. I'll come back.

FROG LOGIC

Blank cards overleaf — write your own in the same shape.

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Access Needs Cards

BLANKS — WRITE YOUR OWN

FROG LOGIC

FROG LOGIC

FROG LOGIC

FROG LOGIC

FROG LOGIC

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You get to decide who needs to know what.

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When Words Go

FOR WHEN THE WORDS HAVE GONE

Print, cut out, and keep them wherever you'll actually reach for them — a wallet, a phone case, or a standard lanyard card holder. They're sized to fit one. Laminating makes them last; a hole punch in the corner puts them on a keyring.

I can't speak right now.

I can hear and understand you. Please give me a moment — I'm not ignoring you.

FROG LOGIC

Please could you help me find something?

I'll show you on my phone, or point.

FROG LOGIC

Where is the toilet?

Pointing is fine. Thank you.

FROG LOGIC

Is there somewhere quieter I can sit?

Just for a few minutes. I'll be alright.

FROG LOGIC

Please could you write that down?

I process written words far better than spoken ones.

FROG LOGIC

I need to leave now.

Nothing is wrong with your service. Please let me past.

FROG LOGIC

CUT ALONG THE OUTLINES · 85 × 54 MM · FITS A STANDARD LANYARD SLEEVE

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When Words Go

SHEET TWO — EXPLAINING AND EVERYDAY

Please don't touch me.

Not personal — touch is painful for me right now.

FROG LOGIC

Please speak slowly and quietly.

Short sentences help. One question at a time.

FROG LOGIC

I'm not being rude.

I'm overloaded. This is what it looks like for me.

FROG LOGIC

I need a moment before I answer.

Silence means I'm processing, not refusing.

FROG LOGIC

I'm autistic.

You don't have to do anything differently unless I ask.

FROG LOGIC

I have ADHD.

I may need instructions repeated or written down.

FROG LOGIC

Please phone this person for me.

Their name and number are on my contact card.

FROG LOGIC

I've already paid.

The receipt is on my phone — one second.

FROG LOGIC

YOU DON'T NEED A DIAGNOSIS, OR ANYONE'S PERMISSION, TO USE THESE

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When Words Go

SHEET THREE — YOURS TO FILL IN

Yes.

FROG LOGIC

No, thank you.

FROG LOGIC

My name is

Best way to communicate with me:

FROG LOGIC

If I need help, please contact

FROG LOGIC

What helps me most

FROG LOGIC

FROG LOGIC

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WRITE YOUR OWN IN THE SAME SHAPE — BIG WORDS, ONE MESSAGE PER CARD

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My Sensory Profile

WHAT I NEED MORE OF, AND LESS OF

> Most sensory advice assumes everyone is the same. Fill this in for you — and note that it can change day to day.

Where I sit, by sense

LEFT = I AVOID IT · RIGHT = I SEEK IT OUT

	AVOID ← → SEEK
Sound	○ ○ ○ ○ ○
Light	○ ○ ○ ○ ○
Touch	○ ○ ○ ○ ○
Smell	○ ○ ○ ○ ○
Taste	○ ○ ○ ○ ○
Movement	○ ○ ○ ○ ○
Temperature	○ ○ ○ ○ ○
Crowds	○ ○ ○ ○ ○

My hard no's

THE ONES THAT END A DAY EARLY

What reliably helps me regulate

In my kit / on my person

- ☐ Ear defenders or loops
- ☐ A stim object
- ☐ Water
- ☐ Sunglasses / cap
- ☐ Familiar snack
- ☐ _____

Knowing this isn't fussiness. It's a maintenance manual.

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How To Help Me

GIVE THIS TO THE PEOPLE WHO KEEP ASKING

> Written to be handed over. Fill in the blanks, or leave it as it is — most of it is true for a lot of us.

When I tell you I'm autistic or ADHD

PLEASE DON'T SAY	TRY INSTEAD
"You don't seem autistic."	"Thanks for telling me."
"Everyone's a bit like that."	"What's it like for you?"
"Have you tried yoga / a diet?"	"Is there anything that would help?"
"But you're doing so well!"	"That must take a lot of effort."
"Isn't everyone on the spectrum?"	"I'll read up on it myself."

In everyday life, it helps if you

- ☐ Send it in writing, not just say it
- ☐ Ask before touching me
- ☐ Let me leave early without a fuss
- ☐ Give notice before plans change
- ☐ Say what you mean, plainly
- ☐ Don't take flat tone personally

If I go quiet or shut down

> Stop talking. Turn the noise and lights down.
>
> Don't ask questions — I can't answer them yet.
>
> Stay nearby but don't crowd me.
>
> Don't try to fix it. It passes.

The specific thing I'd most like you to know

You don't have to understand it all. You just have to believe me.

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My Pond Plan

A PLAN FOR HARD MOMENTS, WRITTEN ON A CALM ONE

> Fill this in when you have the energy — not mid-moment. Change it whenever you learn something new.

Early signs, for me

WHAT SHOWS UP BEFORE A SHUTDOWN OR MELTDOWN

What helps

A PLACE, A SOUND, AN OBJECT, A PERSON, BEING LEFT ALONE

What makes it harder

WORTH NAMING, EVEN JUST FOR YOURSELF

Afterwards, this helps me recover

If someone wants to help

CROSS OUT WHAT DOESN'T FIT — THIS PAGE CAN BE HANDED TO SOMEONE

Please don't touch me without asking first.

Please just sit nearby, quietly — you don't need to fix anything.

Please don't ask me questions right now. I'll talk when I can.

Give me space, and check back in _____ minutes.

Turn the lights or sound down if you're able to.

If I need to reach someone

NAME	HOW TO REACH THEM
NAME	HOW TO REACH THEM

This is allowed to change as you learn more about yourself.

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The Handover Sheet

EVERYTHING THE NEXT PERSON NEEDS TO KNOW

> For respite, a hospital stay, a sitter, a school trip, or the day you're ill. Fill it in once, photocopy it, and it's done. Written to be understood by someone who has never met them.

THEIR NAME / WHAT THEY LIKE TO BE CALLED

MY NAME AND NUMBER

How they communicate

SPEECH, DEVICE, SIGNS, POINTING, WRITING — AND WHAT CHANGES WHEN THEY'RE STRESSED

What a good hour looks like

START HERE — IT'S EASIER THAN A LIST OF DON'TS

Early signs things are going wrong

WHAT YOU'D NOTICE BEFORE ANYONE ELSE WOULD

What to do then — and what NOT to do

PLEASE DO	PLEASE DON'T

Food

SAFE FOODS, ABSOLUTE NO'S, TEXTURES THAT MATTER, HOW IT'S SERVED

Sensory

TOO LOUD / TOO BRIGHT / TOO BUSY MEANS

THINGS THAT RELIABLY HELP

Routine that shouldn't be moved

THE BITS WHERE FLEXIBILITY COSTS MORE THAN IT SAVES

Things that must come with them

- ☐ Ear defenders
- ☐ Comfort item
- ☐ Their device
- ☐ Familiar snack
- ☐ Water bottle
- ☐ Spare clothes

Medical, if relevant

MEDICATION, ALLERGIES, GP, ANYTHING AN AMBULANCE CREW WOULD NEED

If you need to reach someone

FIRST CALL

IF I DON'T ANSWER

Handing this over is not giving up. It's how you get to keep going.

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Appointment Prep

SO THE WORDS ARE READY BEFORE YOU NEED THEM

> Fill this in beforehand and take it with you. Handing someone a piece of paper is a completely valid way to communicate.

WHO I'M SEEING

DATE AND TIME

The main thing I need from this

IF ONLY ONE THING GETS COVERED, THIS IS IT

What's been happening

DATES, FREQUENCY, WHAT CHANGED

Questions I want to ask

What helps me in appointments

- ☐ Written instructions, not verbal
- ☐ Warn me before any touch
- ☐ Plain language, no jargon
- ☐ Time to think before answering
- ☐ I may not make eye contact
- ☐ Someone with me

Afterwards — what was actually said

WRITE IT DOWN BEFORE IT EVAPORATES

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Say It

LINES FOR CONVERSATIONS THAT ARE HARD TO START

Two tones each — Direct for when you're low on energy, Softer for when you want cushioning. Nothing here has to be said word for word.

Cancelling plans

DIRECT

> I need to cancel today — not up for it. Can we find another time?

SOFTER

> I'm sorry, I need to step back from today. It's not about you, I just don't have it in me.

Asking for an accommodation

DIRECT

> I work better with [written instructions / extra time / a quiet space]. Can we set that up?

SOFTER

> Would it be alright if we tried [written instructions / extra time]? It genuinely helps me do better work.

Needing quiet or space

DIRECT

> I need some quiet for a bit. I'll come find you after.

SOFTER

> I hope this doesn't come across wrong — I just need a little space. I'll be back soon.

Explaining, after a hard moment

DIRECT

> That was a shutdown, not me being rude. I needed to stop, not to upset you.

SOFTER

> I owe you some context about earlier — my brain hit a wall and I couldn't keep going.

Asking someone to repeat

DIRECT

> Can you say that again, slower? I lost the middle of it.

SOFTER

> Sorry — could you go back a step? I want to take this in properly.

Saying no

DIRECT

> That doesn't work for me. Thanks for asking though.

SOFTER

> I've thought about it and I'm going to say no this time — but I appreciate you thinking of me.

Correcting a misread tone

DIRECT

> My face doesn't always match what I mean. I'm not annoyed — I'm concentrating.

SOFTER

> Sorry if that came out flat. I'm actually really interested, my expression just doesn't always show it.

Asking for instructions in writing

DIRECT

> Could you send that in a message? I lose things said out loud.

SOFTER

> Would you mind popping that in an email? It helps me get it right first time.

Leaving somewhere early

DIRECT

> I'm going to head off now. It's been good — I've just hit my limit.

SOFTER

> I'm going to slip away shortly. I've really enjoyed it, I just need to go before I'm running on empty.

Declining a phone call

DIRECT

> Could we do this over text instead? Calls are hard for me.

SOFTER

> Would text work? I process written things much better, and I'd rather give you a proper answer.

When plans change suddenly

DIRECT

> I need a minute to adjust — I'd built the day around the old plan.

SOFTER

> That's fine, I just need a moment to rearrange things in my head. Give me five and I'll be with you.

Disclosing, if you choose to

DIRECT

> I'm autistic/ADHD. It mostly means [X], and [Y] helps.

SOFTER

> There's something that might help you understand how I work — I'm [X], which mostly affects [Y].

You don't owe anyone an explanation. These are here for when you want one.

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Ask For It Properly

AN ACCOMMODATIONS REQUEST THAT'S HARD TO SAY NO TO

> Requests land better when they name a specific problem and offer a specific fix. This sheet builds one, line by line, so you're not drafting it from scratch while stressed.

1 — The situation, factually

NO APOLOGY, NO JUSTIFICATION OF YOUR EXISTENCE — JUST WHAT HAPPENS

2 — The effect on the work

THIS IS THE PART THAT GETS IT APPROVED

3 — What I'm asking for

SPECIFIC AND SMALL BEATS VAGUE AND LARGE

4 — What it costs them

USUALLY NOTHING. SAY SO.

Common asks, if you'd rather tick than write

- ☐ Instructions in writing, not verbal
- ☐ Notice before changes to plans
- ☐ Headphones at my desk
- ☐ Fewer, longer blocks of focused time
- ☐ Flexible start time
- ☐ Agenda circulated before meetings
- ☐ Camera off on calls
- ☐ A quieter seat or space
- ☐ Written follow-up after verbal briefs
- ☐ Breaks without justifying each one

Put together, it reads:

"When [1], it means [2]. It would help if [3]. It shouldn't cost anything beyond [4]."

Asking is not failing. It's how the work gets done properly.

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One Task, Broken Down

FOR WHEN THE TASK IS ONE WORD AND STILL IMPOSSIBLE

> "Tidy the kitchen" isn't a task, it's a category. This sheet turns one of those into steps small enough to actually start.

The task, as it currently sits in my head

WHY IT MATTERS (OR WHO IT'S FOR)

WHEN IT ACTUALLY NEEDS DOING

What's the very first physical action?

SOMETHING YOU COULD DO IN UNDER TWO MINUTES, STANDING UP

Then, in order

STOP ADDING STEPS WHEN YOU RUN OUT — THE REST CAN WAIT

- [ ] 1
- [ ] 2
- [ ] 3
- [ ] 4
- [ ] 5
- [ ] 6
- [ ] 7
- [ ] 8
- [ ] 9

What might stop me, and the workaround

THE BLOCKER

WHAT WOULD GET ME PAST IT

Starting badly still counts as starting.

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Low Spoons Day Planner

FOR DAYS WHERE GETTING THROUGH IS THE PLAN

> You don't have to fill in every box today. Blank is a valid answer.

Today is _____

Where my spoons are today

○	○	○	○	○
Empty	Low	Some spare	Steady	Full

The one thing

IF TODAY HAS ROOM FOR EXACTLY ONE THING

Morning

Afternoon

Evening

Brain dump

EVERYTHING ELSE, ALL IN ONE PLACE

Low Spoons Day Planner

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PAGE TWO — THE BODY, NOT THE TO-DO LIST

What my body might need today

- ☐ Water
- ☐ Something to eat
- ☐ A few minutes of quiet
- ☐ Movement, even small
- ☐ A sensory break
- ☐ Rest, actual rest

Gentle wins

NO SCORING, NO STREAKS — ANYTHING THAT WAS OKAY

Tomorrow, one note to myself

If today was hard, that's allowed too.

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Weekly Spoon Tracker

A WEEK OF ENERGY · NO SCORE KEPT

Track where your energy actually goes — no scoring, no streaks, just a picture over a week. Each day, circle where you were, then jot what drained you and what put something back. An empty day is information, not a failure. Print one a week and keep it where you'll see it.

	Empty	Low	Some to spare	Steady	Full	WHAT DRAINED MY SPOONS	WHAT HELPED OR PUT SOME BACK
Monday DATE	○	○	○	○	○		
Tuesday DATE	○	○	○	○	○		
Wednesday DATE	○	○	○	○	○		
Thursday DATE	○	○	○	○	○		
Friday DATE	○	○	○	○	○		
Saturday DATE	○	○	○	○	○		
Sunday DATE	○	○	○	○	○		

This week at a glance

Count how many days landed on each. That's the whole summary — patterns show up on their own after two or three weeks.

Empty	Low	Some to spare	Steady	Full

ONE SHEET A WEEK · CIRCLE, DON'T SCORE · KEEP THE OLD ONES, THEY ADD UP TO SOMETHING

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Permission Cards

CUT OUT. KEEP THE ONES YOU NEED.

Print, cut along the dashed lines, and put them wherever you'll see them — wallet, mirror, desk, back of the phone.

You are allowed to leave early.

FROG LOGIC

You are allowed to not explain why.

FROG LOGIC

Rest is not something you earn first.

FROG LOGIC

You are allowed to need the instructions twice.

FROG LOGIC

Cancelling is kinder than resenting it.

FROG LOGIC

You are allowed to stim in public.

FROG LOGIC

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Permission Cards

SHEET TWO

A quiet day is still a day that counts.

FROG LOGIC

You are allowed to say 'I don't know yet.'

FROG LOGIC

You don't owe anyone eye contact.

FROG LOGIC

You are allowed to want the same meal again.

FROG LOGIC

Not replying yet is not the same as ignoring.

FROG LOGIC

You are allowed to take the easier route.

FROG LOGIC

Blank ones on request — or write your own on the back.

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~~"I'm lazy."~~

↓ frog logic ↓

"I'm conserving pond energy for what matters."

FROG LOGIC

~~"I'm too sensitive."~~

↓ frog logic ↓

"I notice things at a different volume than most."

FROG LOGIC

~~"I can't focus."~~

↓ frog logic ↓

"My attention free-ranges instead of staying fenced in."

FROG LOGIC

~~"I'm so behind."~~

↓ frog logic ↓

"I'm hopping at my own pace, not a race against anyone."

FROG LOGIC

~~"I always mess this up."~~

↓ frog logic ↓

"This is a skill I'm still building, not a verdict on me."

FROG LOGIC

~~"I'm too much."~~

↓ frog logic ↓

"I'm a whole pond's worth of feeling. That's not a flaw."

FROG LOGIC

~~"I should be over this by now."~~

↓ frog logic ↓

"Healing doesn't run on anyone else's clock."

FROG LOGIC

~~"I never finish anything."~~

↓ frog logic ↓

"I start a lot of hops. Some just land later than others."

FROG LOGIC

~~"I'm high-maintenance."~~

↓ frog logic ↓

"I know what my pond needs, and I ask for it."

FROG LOGIC

~~"I overreacted."~~

↓ frog logic ↓

"My reaction matched how big it felt to me, and that's valid."

FROG LOGIC

~~"I'm antisocial."~~

↓ frog logic ↓

"I'm recharging my pond before I hop back out."

FROG LOGIC

~~"I should be able to just push through."~~

↓ frog logic ↓

"Rest is a strategy, not a failure to be tough."

FROG LOGIC

~~"Nobody else struggles with this."~~

↓ frog logic ↓

"Plenty of ponds look calm on the surface and aren't."

FROG LOGIC

~~"I need too much help."~~

↓ frog logic ↓

"Asking for support is how ponds actually work — none of them are solo."

FROG LOGIC

~~"I ruined the day."~~

↓ frog logic ↓

"One hard hour isn't the whole pond's story."

FROG LOGIC

~~"I'm weird."~~

↓ frog logic ↓

"I'm specific. My people will love the specifics."

FROG LOGIC

~~"I can't keep up with everyone."~~

↓ frog logic ↓

"I'm not meant to hop at everyone else's speed."

FROG LOGIC

~~"I have to earn my rest."~~

↓ frog logic ↓

"Rest is allowed on a whim, like a frog just deciding to sit."

FROG LOGIC

🐸

My pace is not a flaw. It's just my pace.

FROG LOGIC

🌿

I am allowed to do things the roundabout way.

FROG LOGIC

🪷

Small hops still move me across the pond.

FROG LOGIC

🐸

I don't have to understand my brain to be kind to it.

FROG LOGIC

💧

Today's version of "enough" counts, whatever size it is.

FROG LOGIC

🌙

Slow days are still days. Ponds don't apologize for being still.

FROG LOGIC

🐸

Rest doesn't have to be earned first.

FROG LOGIC

🌿

My focus moves like water — it doesn't need to be a straight line.

FROG LOGIC

🪷

I'm allowed to take up the whole lily pad.

FROG LOGIC

One small hop today is still forward.

FROG LOGIC

My needs aren't inconvenient — they're just mine.

FROG LOGIC

It's okay to leave the day unfinished.

FROG LOGIC

Different isn't the same as broken.

FROG LOGIC

I can be soft with myself even on a hard day.

FROG LOGIC

Some days are for leaping. Some are for sitting still in the sun.

FROG LOGIC

I get to define what a good day looks like for me.

FROG LOGIC

My brain isn't the problem. It's just the pond I get to know better.

FROG LOGIC

I'm proud of the version of today I actually had.

FROG LOGIC

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After The Diagnosis

NOBODY HANDS YOU THIS PART

> Relief, grief, anger, and blankness are all normal here, sometimes in the same hour. There's no right order and no deadline. Come back to this over months, not days.

What I felt when I found out

TICK FREELY — CONTRADICTORY ONES ARE ALLOWED

- ☐ Relief
- ☐ Grief
- ☐ Anger
- ☐ Numb
- ☐ Vindicated
- ☐ Frightened
- ☐ Doubtful
- ☐ Exhausted
- ☐ Recognised

Things that suddenly made sense

THE MEMORIES THAT REORGANISED THEMSELVES

What I'd say to my younger self

What I want to stop apologising for

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After The Diagnosis

PAGE TWO — WHAT HAPPENS NOW

Who I've told, and how it went

WHO	HOW THEY TOOK IT	AGAIN?

Who doesn't need to know

THIS IS A VALID AND COMPLETE ANSWER FOR MOST PEOPLE

One thing I'm going to do differently

SMALL. JUST ONE.

You haven't changed. The explanation has.

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What I'm Actually Good At

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THE OTHER HALF OF THE ASSESSMENT

> Diagnostic paperwork is a list of deficits. This is the page nobody hands you. Fill it in on a good day and keep it for a bad one.

Things I do better than most people

NOT MODESTLY — ACCURATELY

What people come to me for

A time my brain worked exactly right

THE DETAIL YOU SPOTTED, THE PATTERN YOU CALLED, THE THING YOU FIXED

Traits, reframed

WHAT I'VE BEEN CALLED	WHAT IT ACTUALLY IS
"Obsessive"	
"Too intense"	
"Blunt"	
"Rigid"	
"Oversensitive"	

You are not a list of things that went wrong.

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If you've picked this up because someone you love is autistic — or because you've started to wonder about yourself — welcome. There's nothing here that will make you feel got at.

In plain words

Autism is a different way of being wired, present from birth and lasting for life. It shapes how someone communicates, how they process the world through their senses, and how much predictability they need to feel alright. It isn't an illness, it isn't caused by parenting, and it isn't something that gets cured or grown out of.

What it can feel like from the inside

- Conversation takes deliberate effort that other people seem to get for free.
- Sounds, lights, textures and smells arrive at full volume, all at once, with no filter turning any of it down.
- Sudden changes to plans genuinely hurt, because the old plan had already been built in your head.
- A deep interest isn't a hobby — it's a place to go that reliably feels good.
- Being social is possible, and often wanted, but it costs energy that has to be paid back later.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"You don't look autistic."	There's no look. Many people, especially women and girls, have spent decades learning to hide it.
"Everyone's a bit autistic."	Everyone has a bad day with noise. Not everyone reorganises their whole life around it.
"They're in their own world."	Usually they're in this one, receiving far more of it than you are.
"They don't want friends."	Most do, very much. The usual routes to friendship are just exhausting.
"High or low functioning."	Support needs move about depending on the day, the place and the energy left.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Noticing the detail everyone else walked past.

Deep, sustained knowledge of the things that matter to them.

Honesty you can rely on — you'll know where you stand.

Strong sense of fairness, and the nerve to say so.

Loyalty that doesn't need topping up with small talk.

What actually helps

- ☐ Say what you mean, plainly
- ☐ Send it in writing as well as saying it
- ☐ Ask before touching
- ☐ Give notice before plans change
- ☐ Let them leave early without a fuss
- ☐ Accept flat tone isn't rudeness

If someone tells you this is them

HELPFUL

“Thanks for telling me.”

“What would help?”

“I'll read up on it myself.”

LESS HELPFUL

“But you're so normal!”

“Have you tried a special diet?”

“We all get overwhelmed.”

Nothing here needs fixing. Understanding is the whole job.

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ADHD is one of the worst-named conditions going. It isn't a shortage of attention — it's attention that won't take instructions about where to point.

In plain words

ADHD affects the brain's ability to regulate attention, activity and impulses. Attention isn't missing; it's driven by interest, urgency and novelty rather than by importance. That's why someone can lose six hours to something fascinating and be unable to start a two-minute task they genuinely care about.

What it can feel like from the inside

- Knowing exactly what needs doing, why it matters, and being completely unable to begin.
- Time behaving strangely — five minutes and four hours feeling much the same from the inside.
- Thoughts arriving faster than they can be said, so sentences get interrupted, including your own.
- Emotions landing at full strength immediately, then passing just as fast.
- Hyperfocus, where the world disappears and you forget to eat, drink, or move.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"It's just laziness."	It takes enormous effort to look this disorganised. The wanting was never the problem.
"Only hyperactive boys have it."	Plenty of people are hyperactive only on the inside, and are diagnosed decades late.
"They could focus if they tried."	They can focus, ferociously — just not reliably on demand.
"It's an excuse."	It's an explanation. Those are different things, and only one of them helps.
"Everyone's forgetful."	The difference is scale, consistency, and the cost it exacts over a lifetime.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Ideas nobody else in the room would have had.

Genuinely excellent in a crisis, when urgency finally matches the wiring.

Hyperfocus that produces a month of work in a weekend.

Warmth, humour, and a talent for making things less dull.

Spotting connections between things that appear unrelated.

What actually helps

- ☐ Break tasks into first physical actions
- ☐ Written instructions, not verbal
- ☐ Don't take interruptions personally
- ☐ Put deadlines on things that don't have them
- ☐ Body doubling — work alongside someone
- ☐ Externalise everything: lists, alarms, visible notes

If someone tells you this is them

HELPFUL

“What's the first step? I'll sit with you.”

“Shall I send that in a message?”

“Tell me about the thing you're into.”

LESS HELPFUL

“Just make a list.”

“Everyone's a bit ADHD.”

“You managed it last time.”

Not a shortage of attention. A different set of rules about where it goes.

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Being both autistic and ADHD isn't simply the two lists added together. It's two nervous systems asking for opposite things at the same time.

In plain words

AuDHD describes people who are both autistic and ADHD — very common, and only formally recognised as possible since 2013. Where autism often seeks routine, sameness and depth, ADHD often seeks novelty, stimulation and change. Living with both means constantly negotiating between them.

What it can feel like from the inside

- Craving routine and being bored senseless by it, simultaneously.
- Needing quiet to think, and needing background noise to start.
- Planning obsessively, then abandoning the plan on impulse.
- Deep special interests that rotate without warning and can't be summoned back.
- Looking inconsistent to other people, and feeling inconsistent to yourself.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They cancel each other out."	They don't. You get both sets of needs, often in direct conflict.
"You can't have both."	You can, and many do. Diagnosing them together was only permitted from 2013.
"They seem fine — organised, even."	The autistic systems can mask the ADHD chaos, at considerable cost.
"Just pick a strategy and stick to it."	Strategies that suit one side often actively wind up the other.
"It's a trendy label."	It's a late correction to decades of people getting only half an explanation.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Structure and spontaneity, available in the same person.

Deep expertise, arrived at unusually fast.

Creative problem-solving with genuine follow-through when interest holds.

Unusual range — comfortable in detail and in the big picture.

Real empathy for people who don't fit tidy categories.

What actually helps

- ☐ Flexible routine, not rigid routine
- ☐ Expect the interest to move — plan for it
- ☐ Rest that doesn't require stillness
- ☐ Several ways to do the same task
- ☐ Reduce demands before adding strategies
- ☐ Forgive the inconsistency, including your own

If someone tells you this is them

HELPFUL

“Which bit is loudest today?”

“Would a different way round help?”

“No pressure either way.”

LESS HELPFUL

“You're contradicting yourself.”

“Pick one.”

“But you liked it last week.”

Two systems, one person, permanently negotiating. That's not inconsistency — it's arithmetic.

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Dyslexia has almost nothing to do with intelligence, and rather more to do with how the brain handles the sounds inside words.

In plain words

Dyslexia is a difference in how the brain processes the sound structure of language, which makes decoding written words slow and effortful. Reading can be learned, and often is — but it stays costly in a way it isn't for other people. Spelling and getting words out on the page are usually harder still.

What it can feel like from the inside

- Reading a paragraph three times and retaining none of it, because all the effort went on decoding.
- Knowing a word perfectly well and being unable to spell it, every single time.
- Reading aloud being a genuine dread, not a mild preference.
- Having the whole idea clearly in your head and losing it on the way to the page.
- Being exhausted by tasks other people describe as light reading.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They see letters backwards."	That's a myth. It's about processing sounds, not reversing shapes.
"They're not very bright."	Dyslexia is unrelated to intelligence. Plenty of dyslexic people are exceptional thinkers.
"More reading practice will fix it."	Unstructured practice mostly builds dread. Structured, phonics-based teaching helps.
"They're just careless."	Proofreading your own work is precisely the thing dyslexia makes hardest.
"They'll grow out of it."	It's lifelong. What changes is the strategies and the confidence.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Big-picture thinking, and spotting patterns others miss.

Strong spatial and visual reasoning.

Excellent verbal reasoning and storytelling.

Creative problem-solving, from years of finding workarounds.

Perseverance built by doing hard things daily since childhood.

What actually helps

- ☐ Offer audio versions of anything long
- ☐ Accept voice notes instead of written ones
- ☐ Use clear fonts and generous spacing
- ☐ Never ask someone to read aloud unprepared
- ☐ Don't correct spelling in casual writing
- ☐ Give extra time as standard, not as a favour

If someone tells you this is them

HELPFUL

“Want me to send that as a voice note?”

“Take whatever time you need.”

“I'll summarise the key bits.”

LESS HELPFUL

“Just sound it out.”

“You spelled that wrong.”

“Can you read this out for us?”

The thinking was never the difficult part. Only the getting it in and out.

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Dyspraxia

A POND GUIDE — ALSO CALLED DCD

Dyspraxia is often reduced to clumsiness, which misses most of it. It affects planning and sequencing — of movement, and frequently of everything else too.

In plain words

Dyspraxia, or Developmental Co-ordination Disorder, affects how the brain plans and co-ordinates movement. Actions that become automatic for most people stay effortful and conscious. It commonly comes with difficulties in organisation, sequencing and spatial judgement, and it doesn't go away in adulthood.

What it can feel like from the inside

- Having to think consciously about movements everyone else does without noticing.
- Being genuinely tired by a day of ordinary physical tasks.
- Knowing left from right, eventually, after working it out each time.
- Bruises whose origin is a complete mystery.
- Dreading anything with steps in a fixed order under time pressure.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They're just clumsy."	Co-ordination is one part. Planning, sequencing and organisation are just as affected.
"Practice will sort it."	Practice helps specific skills. It doesn't make the underlying planning automatic.
"It's a childhood thing."	It's lifelong. Adults simply get better at hiding and routing around it.
"They're not trying."	They're trying considerably harder than you are at the same task.
"They're bad at sport."	Team sport especially. That says nothing about strength, fitness or willingness.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Determination built from doing hard things daily.

Creative workarounds nobody else would think to try.

Strong empathy for anyone struggling.

Excellent strategic thinking — planning ahead is a survival skill here.

Verbal and written strengths that often outpace peers.

What actually helps

- ☐ Allow more time, without comment
- ☐ Give instructions one step at a time
- ☐ Written or diagrammed steps beat demonstrated ones
- ☐ Don't rush or watch them closely
- ☐ Offer to carry the tray
- ☐ Choose seating with room to move

If someone tells you this is them

HELPFUL

"Take your time."

"Shall I grab that?"

"Want the steps written down?"

LESS HELPFUL

"Careful!" (as they're already doing it)

"Butterfingers."

"How did you manage that?"

Not careless. Doing consciously what everyone else does on autopilot.

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Dyscalculia is dyslexia's much less famous sibling, and it gets far less patience — because being bad at maths is treated as a personality quirk rather than a difference in wiring.

In plain words

Dyscalculia affects the brain's basic sense of number and quantity. It isn't maths anxiety, though that usually follows, and it isn't a gap in teaching. Estimating amounts, remembering sequences of digits, judging time and handling money all stay difficult regardless of effort.

What it can feel like from the inside

- Looking at a number and it meaning nothing — no sense of how big it is.
- Counting on your fingers as an adult and feeling ashamed of it.
- Being unable to estimate how long something will take, at all.
- Reading a phone number, then losing it between reading and dialling.
- Panic at the till while trying to work out the change.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They're bad at maths."	It's the underlying number sense, not the maths teaching.
"It's just anxiety."	Anxiety is usually the result of years of struggle, not the cause.
"Everyone finds maths hard."	Most people can tell that 40 is roughly double 20 without working it out.
"They'll manage with a calculator."	Calculators help with sums, not with knowing which sum to do.
"It only affects maths lessons."	It affects time, money, directions, cooking, and turning up when you said you would.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Strong verbal and language reasoning.

Creative and intuitive problem-solving.

Excellent at pattern and narrative thinking.

Highly developed workarounds and practical strategies.

Real empathy for anyone told they're just not trying.

What actually helps

- ☐ Write numbers down, don't say them
- ☐ Say clock times, not durations
- ☐ Don't put them on the bill split
- ☐ Allow calculators and apps without comment
- ☐ Break instructions with numbers into steps
- ☐ Give directions as landmarks, not distances

If someone tells you this is them

HELPFUL	LESS HELPFUL
“I'll text you the number.”	“It's simple, look.”
“Shall I sort the bill?”	“You're an adult!”
“Use whatever helps.”	“Just do it in your head.”

Not carelessness with numbers. A different relationship with quantity altogether.

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If writing by hand has always felt disproportionately hard — physically painful, slow, and never matching what's in your head — there's a name for that.

In plain words

Dysgraphia affects handwriting and written expression. For some it's mainly motor: forming letters is effortful and uncomfortable. For others it's mainly about getting organised thought onto a page. Often it's both, and it frequently travels alongside dyslexia or dyspraxia.

What it can feel like from the inside

- Your hand aching after a page that took other people two minutes.
- Handwriting you can't read back yourself.
- A rich, well-organised idea arriving on paper as three flat sentences.
- Gripping the pen so hard your fingers mark.
- Avoiding anything handwritten — cards, forms, notes — for years.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"Messy handwriting, that's all."	The appearance is the visible bit. The effort and the lost content are the real cost.
"They should practise more."	More handwriting mostly produces more pain and more avoidance.
"They're not trying."	Look at the grip. That's what maximum effort looks like.
"Typing is cheating."	Typing removes an obstacle that has nothing to do with the thinking being assessed.
"They can't write well."	They often write beautifully. Getting it out of the hand is the problem.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Ideas that outstrip what makes it onto the page.

Strong verbal expression — ask them out loud.

Inventive with technology and workarounds.

Persistence, from years of doing the hard version.

Good editors of other people's work.

What actually helps

- ☐ Let them type, always
- ☐ Never mark on handwriting appearance
- ☐ Offer forms digitally
- ☐ Accept voice notes and dictation
- ☐ Provide notes rather than expecting copying
- ☐ Ask for it verbally if it's quicker

If someone tells you this is them

HELPFUL	LESS HELPFUL
“Type it if that's easier.”	“Slow down and be neat.”
“Just tell me and I'll write it.”	“I can't read this.”
“I'll fill the form in.”	“Did you rush it?”

The thinking is all there. It's the last six inches, hand to paper, that costs.

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Tourette's & Tics

A POND GUIDE

Almost everything most people think they know about Tourette's comes from one rare symptom being used as a punchline for forty years.

In plain words

Tourette Syndrome involves both motor and vocal tics — sudden, repeated movements or sounds — present for over a year. Tics aren't voluntary, though many people can hold them briefly at a cost. Involuntary swearing is real but affects only a small minority, roughly one in ten.

What it can feel like from the inside

- A building pressure before a tic that only doing it will release.
- Being able to suppress in public, then a much bigger wave the moment you're home.
- Tics worsening when you're stressed, tired, excited — or being watched.
- People assuming you're doing it deliberately, or for attention.
- Exhaustion from managing something that never stops.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"Tourette's is swearing."	Coprolalia affects around 10%. Most tics are blinking, throat-clearing, shrugging, small sounds.
"They could stop if they wanted."	Suppression is possible briefly, like holding a sneeze, and just as unsustainable.
"They're doing it for attention."	Almost everyone with tics would very much prefer not to be noticed.
"It's just nerves."	It's neurological. Stress worsens it; stress didn't cause it.
"It's funny."	It's frequently painful, and it isn't a bit.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Humour, often sharp, and well-earned.

Resilience from managing something visible in public every day.

Genuine compassion for anyone who stands out.

Creativity — tics travel with a lot of creative minds.

Honesty about difficulty, having never had the option of hiding it.

What actually helps

- ☐ Don't mention or count the tics
- ☐ Carry on the conversation normally
- ☐ Never mimic, even affectionately
- ☐ Don't tell them to relax
- ☐ Offer a quieter space if it helps
- ☐ Let them leave and come back

If someone tells you this is them

HELPFUL

Nothing at all — just carry on.

"Want to sit somewhere quieter?"

"Doesn't bother me."

LESS HELPFUL

"Just relax."

"You did it again."

"Say something rude then!"

Not a joke, not a choice, and rarely what television told you it was.

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OCD is probably the most misused word in this whole vocabulary. It isn't about liking things tidy — it's a distressing loop that the person desperately wants out of.

In plain words

OCD involves obsessions — unwanted, intrusive thoughts that cause real distress — and compulsions, which are things done to relieve that distress. The relief never lasts, which is what makes it a loop. Crucially, the thoughts are the opposite of what the person wants, which is exactly why they're so frightening.

What it can feel like from the inside

- Thoughts arriving that horrify you, precisely because they're against everything you believe.
- Knowing a fear is irrational and being entirely unable to let it go.
- Checking something you already checked, because knowing isn't the same as feeling certain.
- Rituals eating hours you'll never get back.
- Relief lasting about ninety seconds before the doubt returns.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"I'm so OCD about my desk."	Enjoying tidiness is a preference. OCD is distress you'd pay anything to be rid of.
"It's about cleaning."	Contamination is one theme. Many people have no cleaning compulsions whatsoever.
"Just stop doing the ritual."	That's the treatment, roughly — but it takes structured therapy and considerable courage.
"The thoughts mean something."	They mean the opposite. OCD attaches to whatever you'd least want to be true.
"Reassure them and they'll settle."	Reassurance is a compulsion by proxy. It feeds the loop rather than starving it.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Extraordinary attention to detail and consequences.

Deep conscientiousness and care about doing right.

Considerable courage — treatment asks a lot and people do it.

Strong empathy for others in distress.

Sharp analytical thinking, when it isn't turned inward.

What actually helps

- ☐ Don't provide repeated reassurance
- ☐ Take the distress seriously
- ☐ Be patient with slow progress
- ☐ Don't join in with the rituals
- ☐ Support professional help — it works
- ☐ Never say 'I'm a bit OCD'

If someone tells you this is them

HELPFUL

"That sounds exhausting."

"I'm not going to reassure you, but I'm here."

"How can I help without feeding it?"

LESS HELPFUL

"But is it definitely fine?"

"I'm so OCD too!"

"Just don't think about it."

The thoughts are not you. They're the thing happening to you.

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PDA describes a profile where everyday demands feel genuinely threatening — including demands the person actually wants to meet. It's the one on this shelf where even the name is under active debate.

In plain words

PDA usually stands for Pathological Demand Avoidance, though many people in the community prefer Persistent or Pervasive Drive for Autonomy, arguing that 'pathological' stigmatises. Others keep the original, saying it honestly conveys how far outside conscious control it sits. Both views come from people living it. It is not a standalone diagnosis in the DSM; in the UK it's sometimes noted as a demand-avoidant profile within autism.

What it can feel like from the inside

- A request landing like a threat, even a kind one, even one you agree with.
- Wanting to do the thing and being unable to the moment it becomes expected.
- Praise making it worse, because now it's expected again next time.
- Negotiating, deflecting, or going quiet — anything to keep some control.
- For some, none of this shows outwardly at all; it turns inward instead.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They're just defiant."	Defiance is a choice. This is a nervous system reading demands as danger.
"Be firmer with them."	Escalating demands escalates the threat. It reliably makes things worse.
"They manage at school, so it's parenting."	Holding it together all day is why it collapses at home, where it's safe.
"They can't be, they're so easy-going."	Internalised PDA is quieter and often missed entirely.
"It's not a real thing."	It isn't a formal DSM diagnosis. It's still a useful description of a real pattern.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Strong sense of justice and personal autonomy.

Creative, imaginative, often socially perceptive.

Excellent negotiators, from constant practice.

Thrive genuinely when given real choice and control.

Question rules that deserve questioning.

What actually helps

- ☐ Offer choices, not instructions
- ☐ Reduce demands before adding strategies
- ☐ Drop praise that raises expectation
- ☐ Phrase things indirectly ('I wonder if…')
- ☐ Give control over timing and order
- ☐ Treat collapse as distress, not defiance

If someone tells you this is them

HELPFUL	LESS HELPFUL
“Whenever you're ready.”	“You need to do this now.”
“Would you rather A or B?”	“Because I said so.”
“No pressure — genuinely.”	“See, you managed it!”

Autonomy isn't a luxury here. It's the thing that makes everything else possible.

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Sensory Differences

A POND GUIDE

Most people know about the five senses. Sensory processing differences involve eight — and any of them can arrive far too loud, far too quiet, or unpredictably both.

In plain words

Sensory processing describes how the brain receives and interprets information from the senses — including balance, body position, and internal signals like hunger or needing the loo. Some people are hypersensitive, receiving too much; some are hyposensitive, receiving too little and seeking more. Many are both, in different senses, on different days.

What it can feel like from the inside

- A supermarket being genuinely painful rather than mildly annoying.
- A clothing label feeling like it's actively attacking you.
- Craving deep pressure, loud music or strong flavours to feel switched on.
- Not registering hunger, thirst or cold until it's become urgent.
- The same environment being fine on Tuesday and unbearable on Thursday.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They're being fussy."	The distress is real and physical. Nobody chooses this over a comfortable jumper.
"They'll get used to it."	Forced exposure usually sensitises further rather than habituating.
"It's only about noise."	Eight senses, including balance, body position and internal signals.
"They were fine last week."	Sensory tolerance moves with tiredness, stress, hormones and illness.
"Ear defenders are antisocial."	They're the reason the person can stay at all.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Noticing things nobody else registers.

Deep enjoyment of textures, sounds and tastes others find ordinary.

Excellent early warning for problems — the smell, the sound, the flicker.

Strong aesthetic sense and attention to environment.

Real skill at designing spaces that work for people.

What actually helps

- ☐ Ask before changing the environment
- ☐ Offer a quieter room as standard
- ☐ Warn before loud or bright things
- ☐ Let them wear the ear defenders
- ☐ Don't comment on food or clothing choices
- ☐ Believe them the first time

If someone tells you this is them

HELPFUL

"Is this alright in here?"

"Want the lights down?"

"We can go whenever."

LESS HELPFUL

"It's not that loud."

"You'll get used to it."

"Just try a bit."

Not fussy. Receiving a different amount of the world than you are.

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Bipolar is a difference in how the brain regulates mood and energy — not moodiness, and not a personality flaw. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Bipolar involves episodes of elevated mood and energy (mania or the milder hypomania) alternating with periods of depression. Episodes last weeks or months, not hours, and they aren't triggered by ordinary good or bad days. It's lifelong, and is said to be manageable with the right treatment and support.

What it can feel like from the inside

- Energy and confidence arriving in a wave that feels wonderful and is genuinely dangerous.
- Sleeping three hours and feeling brilliant — the earliest reliable warning sign.
- Making plans and purchases during a high that make no sense afterwards.
- Then a flatness that isn't sadness so much as everything being switched off.
- Some people have described grieving the version of themselves that existed during the high.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"Everyone has mood swings."	Mood swings last hours. Episodes last weeks, and reorganise your whole life.
"Mania is just being happy."	It impairs judgement, sleep and safety. Plenty of it is frightening rather than fun.
"They're fine now, so it's over."	Stable is the goal and often achieved. It doesn't mean it's gone.
"They should come off the medication."	Stopping is the most common route back to crisis. Never advise this.
"It's so bipolar of the weather."	Using it as a synonym for changeable makes the real thing harder to disclose.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Genuine creativity and originality of thought.

Deep empathy for people at the bottom of things.

Considerable resilience — this takes real strength to manage.

Insight into their own mind that most people never develop.

Energy and drive that, well managed, achieves a great deal.

What actually helps

- ☐ Learn their early warning signs with them
- ☐ Support the treatment, don't second-guess it
- ☐ Stay in touch during the low ones
- ☐ Take sleep changes seriously
- ☐ Don't make big decisions during a high
- ☐ Treat stability as an achievement

If someone tells you this is them

HELPFUL

"I've noticed you're sleeping less — how are you doing?"

"I'm around either way."

"What helps when it's like this?"

LESS HELPFUL

"You seem manic!" (as a joke)

"Do you still need those tablets?"

"Snap out of it."

Not moodiness. A different system for regulating energy, which can be managed well.

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FROG LOGIC — MADE WITH CARE

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Complex PTSD comes from harm that repeated over time, usually when you were too young or too trapped to leave. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

CPTSD develops after prolonged, repeated trauma rather than a single event. Alongside the usual trauma symptoms it involves difficulties with emotional regulation, a persistently negative self-view, and trouble in relationships. When it happens during childhood it shapes brain development — which is why many people describe it as a form of neurodivergence rather than simply an illness.

What it can feel like from the inside

- Reacting to something small with a force that surprises even you.
- Scanning every room and every face for signs something's about to go wrong.
- Apologising constantly, for existing as much as anything.
- Going numb and distant when things get difficult, without choosing to.
- Struggling to believe good things or kind people will stay.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"That was years ago."	Time doesn't undo how a developing brain adapted to survive.
"It wasn't that bad."	CPTSD comes from repetition and inescapability, not from single dramatic events.
"They're being dramatic."	A nervous system trained for danger reacts as if there's danger. That's not performance.
"They should just move on."	Recovery is real and possible, but it's therapeutic work, not a decision.
"It's the same as PTSD."	It overlaps, but adds lasting effects on self-image, emotions and relationships.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Extraordinary ability to read a room and its moods.

Deep empathy, often for people others overlook.

Practical competence built out of necessity, very young.

Fierce protectiveness of anyone vulnerable.

Real courage — recovery asks a great deal and people do it anyway.

What actually helps

- ☐ Be consistent and predictable
- ☐ Give warning before change
- ☐ Let them leave any situation freely
- ☐ Say what you mean, and mean it
- ☐ Don't push for the story
- ☐ Support trauma-informed therapy

If someone tells you this is them

HELPFUL

"That makes sense, given everything."

"You're safe here."

"Take as long as you need."

LESS HELPFUL

"But that was ages ago."

"Other people had it worse."

"Have you tried forgiving them?"

Your nervous system learned to keep you alive. It's allowed to take a while to unlearn it.

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Few labels carry as much unfair stigma as this one, including from professionals who should know better. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you. Many people prefer 'emotional intensity' or 'emotionally unstable personality disorder', and some reject the label entirely.

In plain words

BPD involves intense emotions that arrive fast, run hot, and take longer than usual to come down; a fear of abandonment; and an unstable sense of self. It often follows childhood trauma, though not in every case, and it overlaps heavily with autism and ADHD — many women in particular are given this label when autism would fit better.

What it can feel like from the inside

- Feeling everything at maximum volume, then being told you're overreacting.
- A small change in someone's tone reading as certain rejection.
- Loving people intensely and being terrified they'll leave.
- Not being sure who you are when you're on your own.
- Emotions passing eventually — and the shame afterwards lasting far longer.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They're manipulative."	That word has done enormous damage. It's distress, communicated badly because nothing else worked.
"It's untreatable."	It has one of the better recovery rates of any diagnosis, especially with DBT.
"They're attention seeking."	Feeling a need for deep connection, showing as distress when nothing else has worked.
"It's a personality flaw."	In some cases, though not all, an adaptation to an environment that wasn't safe.
"It is said that women get this, men get PTSD."	A criticism raised from within the field itself, and one reason the label is so contested.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Emotional depth and honesty most people never reach.

Extraordinary empathy — they feel it with you.

Loyalty and passion in relationships.

Perceptiveness about what people are really feeling.

Real capacity for change — this is a diagnosis people genuinely recover from.

What actually helps

- ☐ Be consistent — say what you'll do, then do it
- ☐ Validate the feeling before the facts
- ☐ Support DBT or similar therapy
- ☐ Don't withdraw suddenly without explanation
- ☐ Keep boundaries kind and predictable
- ☐ Never use 'manipulative'

If someone tells you this is them

HELPFUL	LESS HELPFUL
“That sounds really painful.”	“You're being manipulative.”
“I'm not going anywhere.”	“Calm down.”
“I'll message you at six, and I will.”	“Here we go again.”

Intensity is not a character defect. And this is one people do recover from.

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Anxiety is often the *result* of being neurodivergent in a world that isn't built for you, rather than a separate thing. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Anxiety disorders involve worry and physical alarm that persist beyond the situation causing them. The body's threat system runs hot and doesn't switch off. Social anxiety specifically involves intense fear of judgement — and for a lot of neurodivergent people, it's a very reasonable response to years of getting it subtly wrong in public.

What it can feel like from the inside

- Your body reacting to danger while your mind knows perfectly well there isn't any.
- Replaying a two-second interaction from four years ago at three in the morning.
- Avoiding something small, then feeling ashamed of the avoidance.
- Exhaustion — the alarm running all day takes real fuel.
- Being told to relax, which has never once worked for anybody.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"Just don't worry about it."	If that worked, nobody would have anxiety. The worry isn't chosen.
"It's all in your head."	It's substantially in your body — heart, gut, chest, muscles.
"Everyone gets nervous."	Nervous passes. This doesn't, and it makes decisions for you.
"They're being antisocial."	Social anxiety is usually wanting to be there and finding it unbearable.
"Avoiding it will help."	Avoidance shrinks the world. Gradual, supported exposure is what actually works.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Excellent risk assessment — they've thought of the thing you missed.

Deep conscientiousness and preparation.

Empathy for anyone frightened or out of their depth.

Attention to detail and consequence.

Considerable courage, since they do things afraid rather than unafraid.

What actually helps

- ☐ Give notice and detail in advance
- ☐ Don't repeatedly reassure — support instead
- ☐ Go with them the first time
- ☐ Offer an exit route and mean it
- ☐ Say what will happen, specifically
- ☐ Never say 'just relax'

If someone tells you this is them

HELPFUL	LESS HELPFUL
“What would make this easier?”	“Just relax.”
“We can leave whenever.”	“There's nothing to worry about.”
“Here's exactly what'll happen.”	“You're overthinking it.”

The alarm is real, even when the danger isn't. Turning it down takes support, not willpower.

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Depression among neurodivergent people is very often the accumulated cost of years of masking and unmet needs, rather than something arriving out of nowhere. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Depression is a persistent lowering of mood, energy, interest and self-worth that lasts weeks or longer and doesn't lift with rest or good news. It's a medical condition, not a mood or an attitude. For many neurodivergent people it's tangled up with burnout, and treating both matters.

What it can feel like from the inside

- Not sadness so much as the volume turned down on everything, including sadness.
- Things you love feeling like admin.
- Being tired in a way sleep doesn't touch.
- Small tasks — a shower, a reply — becoming genuinely enormous.
- Knowing exactly how irrational the self-criticism is, and believing it anyway.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"Cheer up, it might never happen."	Depression isn't about circumstances, which is what makes it so hard to argue with.
"They're being lazy."	Effort is the thing depression removes. What you're seeing is the illness.
"Just get some exercise."	Exercise genuinely helps and is also nearly impossible mid-episode. Both are true.
"They've got nothing to be depressed about."	It doesn't require a reason. It isn't a response to your assessment of their life.
"It's just burnout."	They overlap heavily for neurodivergent people. Worth treating both, not choosing.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Depth of thought and honesty about hard things.

Compassion for others struggling — no performance required.

Realism that can be genuinely useful.

Considerable endurance; getting through a day like this is not nothing.

Insight into their own mind, hard won.

What actually helps

- ☐ Turn up without requiring anything
- ☐ Keep inviting, even after refusals
- ☐ Notice small things without praising loudly
- ☐ Do the task with them, not for them
- ☐ Support treatment — it works
- ☐ Ask directly how bad it is

If someone tells you this is them

HELPFUL

"I'm coming round Tuesday, no need to tidy."

"Shall we do it together?"

"How bad is it, honestly?"

LESS HELPFUL

"Others have it worse."

"You just need a holiday."

"Have you tried being positive?"

You are not the illness. And this is treatable — genuinely, reliably treatable.

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If certain small sounds provoke instant, disproportionate rage or panic in you, you are not being difficult and you are far from alone. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Misophonia is a strongly reduced tolerance to specific sounds — most often chewing, breathing, sniffing, tapping, pen-clicking. Triggers provoke immediate anger, panic or disgust that the person knows is out of proportion and cannot stop. Brain imaging suggests real differences in how these sounds are processed. It affects a substantial minority of people and commonly starts in childhood.

What it can feel like from the inside

- A sound arriving like an assault, with rage or panic before any thought.
- Being unable to hear anything else once you've noticed it.
- Feeling ridiculous and ashamed of a reaction you can't control.
- Avoiding meals with people you love.
- Being told you're overreacting by everyone, for years.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"Everyone hates chewing sounds."	Mild annoyance isn't the same as a full fight-or-flight response.
"You're being rude."	The reaction arrives before any decision. Leaving the room is the polite version.
"Just ignore it."	The difficulty disengaging attention is the core of the condition.
"It's not a real thing."	It's increasingly well evidenced, including on brain imaging.
"They'll grow out of it."	It usually starts in childhood and often persists. Management helps; growing out doesn't.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Exceptional attention to sound and detail.

Often musical, or acutely attuned to tone and rhythm.

Strong self-awareness about their own reactions.

Considerable self-control — most people mask this for years.

Empathy for anyone with an invisible difficulty.

What actually helps

- ☐ Don't take the reaction personally
- ☐ Eat with the telly or music on
- ☐ Give them a seat furthest from noise
- ☐ Let them wear headphones at meals
- ☐ Don't tap, click or crunch deliberately
- ☐ Believe them the first time

If someone tells you this is them

HELPFUL

“Tell me if I'm doing it.”

“Shall I put some music on?”

“Sit wherever's easiest.”

LESS HELPFUL

“It's only eating.”

“You're so sensitive.”

doing it on purpose to tease

The rage isn't aimed at you. It arrives before anyone gets a say in it.

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Aphantasia

A POND GUIDE — CONTESTED GROUND

Some people picture an apple and see an apple. Some see nothing at all, and only discover in adulthood that 'picture it in your mind' was never a figure of speech. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Aphantasia is the absence of voluntary visual imagery — no mental pictures. Around one to four percent of people have it. The opposite, hyperphantasia, means imagery as vivid as real seeing. Neither is a disorder; both are variations. Many people with aphantasia have excellent memory and imagination, just not in pictures.

What it can feel like from the inside

- Realising in your thirties that everyone else was being literal.
- Remembering facts about a face perfectly without being able to see it.
- Guided meditation being completely useless.
- Reading a novel and building the world in concepts rather than images.
- Being unable to conjure a lost person's face, which can hurt a great deal.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"You must have a bad memory."	Memory is often fine. It's stored as facts and relationships rather than pictures.
"You have no imagination."	Plenty of aphantasic people are artists, writers and designers. Imagination isn't only visual.
"You just aren't trying."	There's no effort that produces the picture. It isn't a skill being neglected.
"It's a disorder."	It's a variation. Most people who have it are perfectly fine and often unaware.
"Then you don't dream."	Many do dream in images. Voluntary imagery is the part that's absent.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Thinking in concepts and structure rather than pictures.

Often less troubled by intrusive visual memories.

Strong verbal and logical reasoning.

Precise, factual recall.

A very unusual perspective in creative work.

What actually helps

- ☐ Describe things in words, not 'picture it'
- ☐ Skip visualisation exercises
- ☐ Use lists rather than mental walkthroughs
- ☐ Give diagrams and written detail
- ☐ Don't quiz them on what they can see
- ☐ Take their word for it

If someone tells you this is them

HELPFUL

“That's fascinating — how do you do it instead?”

“I'll write it down.”

“Makes sense.”

LESS HELPFUL

“Try harder, you'll see it.”

“That's so sad.”

“So what happens when you read?” *for the twentieth time*

Not a missing picture. A different filing system entirely.

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If Tuesday has always been yellow-green and you assumed everyone knew that, this one's for you. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Synaesthesia is an automatic crossing of the senses — letters and numbers having colours, sounds having shapes, days of the week having positions in space, words having tastes. It's consistent over a lifetime, involuntary, and present in perhaps four percent of people. It isn't imagination and it isn't a symptom of anything.

What it can feel like from the inside

- Assuming until adulthood that everyone's alphabet was coloured.
- Someone spelling a name wrong and it looking the wrong colour.
- Music arriving with texture and shape as well as sound.
- Being certain about associations you can't possibly justify.
- Finding the whole thing pleasant, mostly — this one's rarely a burden.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"You're imagining it."	It's involuntary and remarkably consistent — testable across decades.
"It's drug related."	Congenital synaesthesia has been there since childhood.
"Everyone associates things."	Learned association is different from automatic perception.
"It must be distracting."	Most synaesthetes wouldn't give it up. It often aids memory.
"It's a disorder."	It's a perceptual variation, and generally a rather nice one.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Unusually strong memory — the extra channel helps.

Creative and artistic advantages.

Rich, textured experience of ordinary things.

Often excellent with names, dates and sequences.

A genuinely unusual perspective on how minds can work.

What actually helps

- ☐ Take it seriously when they describe it
- ☐ Ask what it's actually like
- ☐ Don't tell them they're wrong about it
- ☐ Don't test them like a party trick
- ☐ Accept their associations are fixed
- ☐ Enjoy it with them

If someone tells you this is them

HELPFUL

“What colour is my name?” *asked once, kindly*

“That sounds lovely.”

“How does that work?”

LESS HELPFUL

“Prove it.”

“That's not a real thing.”

“Do it again for my friend.”

Extra channels, permanently on. Mostly a good deal.

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Alexithymia isn't the absence of feelings. It's having them at full strength with no reliable way to identify or name them. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Alexithymia means difficulty identifying and describing your own emotions. Physical sensations are noticed; the label attached to them isn't. It's common alongside autism — roughly half of autistic people have it — and it often explains things wrongly attributed to coldness or a lack of empathy.

What it can feel like from the inside

- Knowing something is wrong and having absolutely no idea what.
- Answering 'how do you feel?' with a description of your day instead.
- Realising three days later that you were furious on Tuesday.
- Feeling it in your stomach, your chest, your jaw — never in words.
- Being called unemotional while feeling a great deal, silently.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They don't have feelings."	They have them fully. The naming is what's missing.
"They lack empathy."	Often the opposite — they absorb others' emotions and can't process their own.
"They're being evasive."	"I don't know" is frequently the whole, honest truth.
"It's the same as being autistic."	It commonly co-occurs but is separate, and plenty of non-autistic people have it.
"They just need to open up."	The information isn't being withheld. It hasn't been decoded yet.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Steadiness in a crisis when others are overwhelmed.

Fair, unemotional judgement where that's needed.

Deep loyalty, shown in actions rather than words.

Strong observational skills about other people.

Honesty — no performance of feelings they can't locate.

What actually helps

- ☐ Ask about the body, not the feeling
- ☐ Give time — days, sometimes
- ☐ Watch actions rather than words
- ☐ Offer options: 'angry, or tired, or hungry?'
- ☐ Accept 'I don't know' as complete
- ☐ Don't demand emotional performance

If someone tells you this is them

HELPFUL

“Where do you feel it?”

“No rush — tell me when you know.”

“Is it more like A or B?”

LESS HELPFUL

“You must know how you feel.”

“Why won't you open up?”

“You're so cold.”

The feelings are all there. It's the labels that didn't come with them.

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If you've ever walked past your own colleague, or failed to recognise a friend out of context, and quietly built a life around covering for it — there's a name for that. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Prosopagnosia, or face blindness, is difficulty recognising faces, including very familiar ones. Developmental prosopagnosia is present from birth and affects around two percent of people. It has nothing to do with memory, attention or caring — the face-processing system simply works differently. Most people compensate using voice, hair, gait and context.

What it can feel like from the inside

- Recognising people by coat, voice or walk, and being lost when they change any of it.
- Watching a film and losing track of which character is which.
- Greeting everyone warmly as insurance, in case you know them.
- Not recognising your own face in a group photograph.
- Being thought rude or standoffish for years.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"You're bad with names."	Different thing. Names can be perfect; the face is what won't attach.
"You weren't paying attention."	Attention doesn't help. The processing happens elsewhere.
"You don't care about people."	Frequently the reverse — the anxiety about causing offence is constant.
"Everyone forgets faces sometimes."	Not their own family's, reliably, forever.
"You recognised me last week!"	Context did that. Same person, different setting, different outcome.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Strong recognition of voice, gait and mannerism.

Excellent contextual and situational memory.

Judges people on behaviour rather than appearance.

Highly developed social workarounds.

Genuinely unbiased about looks.

What actually helps

- ☐ Say your name when you greet them
- ☐ Keep hair, glasses, coat predictable if you can
- ☐ Don't take being missed personally
- ☐ Don't test them or make it a joke
- ☐ Introduce yourself again, cheerfully
- ☐ Mention who's in the room quietly

If someone tells you this is them

HELPFUL

"Hi, it's me, Dave."

"No bother at all."

"Sarah's the one in blue."

LESS HELPFUL

"Guess who!"

"You don't remember me?"

"How can you not know my face?"

Not rudeness, not carelessness. A face-shaped gap in an otherwise fine memory.

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Often still called selective mutism, which is unfortunate — nothing about it is selected. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Situational mutism is an inability to speak in certain settings despite speaking freely in others. It's an anxiety-based freeze response, not shyness, refusal or defiance. It usually begins in childhood and can persist into adult life. Many autistic people also lose speech temporarily during overload, which is related but distinct.

What it can feel like from the inside

- Wanting to answer, knowing the answer, and no sound coming out at all.
- Your throat physically closing rather than choosing silence.
- Speaking freely at home and being mute at the school gate.
- Adults treating your silence as rudeness or stubbornness.
- The pressure to speak making it categorically worse.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They're just shy."	Shy people speak eventually. This is a freeze response with a physical component.
"They're being difficult."	Nobody chooses this. It's frightening and isolating for the person in it.
"Make them answer."	Pressure is the single most reliable way to make it worse.
"They can talk at home, so they can here."	That's the definition, not a contradiction.
"They'll grow out of it."	Some do; many need support. Waiting alone isn't a plan.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Exceptional listening and observation.

Rich inner life and often strong writing.

Deep loyalty to the people they can speak with.

Sensitivity to atmosphere and other people's discomfort.

Considerable courage every time they try.

What actually helps

- ☐ Never insist on a verbal answer
- ☐ Talk alongside them without requiring replies
- ☐ Reduce audience size
- ☐ Accept writing, typing, nodding, pointing
- ☐ Don't announce or praise when they do speak
- ☐ Give questions in advance

If someone tells you this is them

HELPFUL

"Nod if that's right."

"Write it down if you'd rather."

"No rush."

LESS HELPFUL

"Cat got your tongue?"

"She spoke! Everyone, listen!"

"Just say it."

Not selected, not shyness, not rudeness. A freeze that needs patience, not pressure.

A friendly explainer written for the community — not a diagnostic tool and not medical advice. If you want an assessment, your GP is the place to start. Nobody experiences this identically, so take what fits and leave the rest.

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Auditory Processing

A POND GUIDE — CONTESTED GROUND

Your hearing test comes back perfect, and yet you still can't follow a conversation in a pub. Both of those things can be true. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you.

In plain words

Auditory Processing Disorder affects how the brain interprets sound rather than how the ear detects it. Hearing is typically normal. What's difficult is separating speech from background noise, processing fast speech, and holding spoken instructions long enough to act on them. It commonly travels with ADHD, autism and dyslexia.

What it can feel like from the inside

- Hearing every word and assembling the meaning several seconds late.
- A busy café turning speech into an unsortable wall of sound.
- Saying 'what?' then answering before they repeat it.
- Mishearing similar-sounding words constantly.
- Being exhausted by a day of listening, because it's active work.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"Get your ears tested."	Already done. They're fine. That's rather the point.
"You weren't listening."	Listening was happening at maximum effort the whole time.
"I'll just say it louder."	Volume isn't the problem. Clarity, pace and background noise are.
"You heard me before."	Quiet room versus noisy room is an enormous difference here.
"They're away with the fairies."	They're two sentences behind, catching up.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Excellent visual and written comprehension.

Strong pattern-spotting in text.

Genuine empathy for anyone struggling to follow.

Highly developed strategies for checking and confirming.

Often unusually good at reading faces and body language.

What actually helps

- ☐ Get their attention before speaking
- ☐ Slower and clearer beats louder
- ☐ Choose quiet places to talk
- ☐ Face them, don't call from another room
- ☐ Back it up in writing
- ☐ Don't mind repeating

If someone tells you this is them

HELPFUL

“Shall I text you that?”

“Let's sit somewhere quieter.”

“No problem, I'll say it again.”

LESS HELPFUL

“ARE YOU DEAF?”

“I've told you three times.”

“You only hear what you want to.”

The ears are fine. It's the sorting office that's under pressure.

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